

Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°083/MVE B_05/08/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (53)	- Haut-Uélé (6/13): Boma Mangbetu, Gombari, Isiro, Pawa, Rungu and Wamba - Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. - North Kivu (12/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka, Musienene and Lubero. - South Kivu (1/34): Miti-Murhesa - Tshopo (6/23): Bafwasende, Kabondo, Lubunga, Makiso-Kisangani, Mangobo and Wanie-Rukula
Reporting date	August 5, 2026
Publication date	August 6, 2026

KEY INDICATORS — CUMULATIVE NUMBERS AS OF AUGUST 5, 2026

CONFIRMED CASES — 5 PROVINCES 4053 +80 new cases (Ituri 61 · N-Kivu 11 · Haut-Uélé 7 · Tshopo 1)	CONFIRMED DEATHS · LEATHITIS 1850 · (45.7%) +49 deaths today (Ituri 38 · N-Kivu 10 · H-Uélé 1)	SUSPECTED CASES OF THE DAY 282 Ituri 193 · N-Kivu 86 · Haut-Uélé ND · Tshopo 2 · South Kivu 1
PATIENTS IN ISOLATION 694 Ituri 468 · N-Kivu 205 · Haut-Uélé ND · Tshopo 4 · South Kivu 17	CURED — CUMULATIVE 793 +17 healed (Ituri 15, Haut Uele 2)	NATIONAL CONTACT TRACKING 77.7% 14,673 / 18,886 views · Ituri 79.3% · N-Kivu 76.9% · H-Uélé 53.2% · Tshopo 80.0% · South Kivu 100%

1. Epidemiological Highlights

- Epidemiological situation:** Two new health zones have been affected in the last 24 hours: Gombari (Haut-Uélé), with an imported case from PK-51 Niania (Ituri) followed by a death (DSE completed), and Bafwase²nde (Tshopo), where the 8th provincial case was confirmed in the Baego health area. The epidemic now extends to 53 out of 140 health zones, spread across five provinces: Ituri (28/36), North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23), and South Kivu (1/34).
- Case trends:** In the last 24 hours, 80 new confirmed cases, 49 confirmed deaths, and 17 recoveries have been recorded. Today's deaths include 33 community deaths confirmed by post-mortem testing (66.0%) and 16 deaths occurring in treatment centers.

In **Ituri**, 61 new confirmed cases were reported by the laboratory, identified in Bunia (17), Rwampara (16), Bambu (8), Nizi (6), Fataki (5), Mongbwalu (3), Nia-Nia (2), Aru (1), Komanda (1), Lita (1), and Gethy (1), along with 38 deaths, 27 of which were community deaths (71.1%). The 11 deaths that occurred in treatment centers were distributed as follows: Bunia (5), Mongbwalu (2), Komanda (2), Fataki (1), and Nizi (1). Nine (9) positive repeat tests were also reported.

North **Kivu**: 11 new confirmed cases in Katwa (8), Musienene (2) and Beni (1), and 10 deaths, 5 among the new cases confirmed cases (Katwa 4, Musienene 1) and 5 occurring in CTE (Katwa 2, Butembo 2, Lubero 1).

o **Haut-Uélé** : 7 new confirmed cases in Pawa (3), Boma Mangbetu (2), Isiro (1) and Gombari (1), including 6 living cases and 1 death confirmed by swab. Gombari becomes the 6th affected health zone in the province, the case having been imported from PK-51 Niania (Ituri).

o **Tshopo** : 1 new confirmed case has been reported in the Bafwasende health zone, bringing the provincial total to 8 confirmed cases, 5 deaths and 1 recovery, representing a case fatality rate of 62.5%. The patient was transferred to the Ebola Treatment Center (ETC) at the Kisangani University Hospital.

o **South Kivu** : The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries, in addition to 2 cases probable. The province has gone 57 days without a new confirmed case since June 10, 2026.

- **Analysis**: Ituri remains the epicenter with 86.9% of cumulative cases and 76.3% of daily confirmations, and its case fatality rate (42.7%) remains significantly lower than that of North Kivu (67.6%), which accounts for 13.8% of new cases. Nationally, 33 of the 49 daily deaths (67.3%) occurred in the community, compared to 71.2% the previous day. Contact tracing has risen to 77.7%, up from 75.3% the previous day, but remains well below the operational threshold of 95%. The expansion to two new health zones in 24 hours, in two separate provinces both linked to the Niania axis, marks a geographical spread after several days of stability.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 53 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.



Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health areas	New cases (24h)
Ituri	3,521	1,502	42.7%	28/36 (77.8%)	61
North Kivu	447	302	67.6%	12/34 (35.3%)	11
Haut-Uélé	74	41	55.4%	6/13 (46.2%)	7
Tshopo	8	5	62.5%	6/23 (26.1%)	1
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	4053	1,851	45.7%	53 out of 140	80

Case fatalities recalculated from the day's totals. Two new health zones (Gombari, Haut-Uélé; Bafwasende, Tshopo) have been affected in the last 24 hours.

Ituri province accounts for 86.9% of confirmed cases (n = 3521) and 81.1% of deaths (n = 1505), with a case fatality rate of 42.7%. The health zones of Bunia (967 cases), Rwampara (708), Mongbwalu (561), Nizi (437), Lita (152), and Nyankunde (115) represent 83.4% of the cases reported at the provincial level and 72.5% of the national total. North Kivu, the second-largest outbreak hotspot, recorded 11.0% of new cases in a single day and maintains a significantly higher case fatality rate than Ituri.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,521	1,502	42.7%	61	27	11	38
Adja	11	1	9.1%	0			
Aru	6	6	100.0%	1	1		1
Ariwara	7	2	28.6%	0			
Aungba	10	5	50.0%	0			
Bamboo	77	20	26.0%	8	3		3
Boga	1	1	100.0%	0			
Bunia	967	297	30.7%	17	3	5	8
Damascus	18	8	44.4%	0			
Drodro	11	6	54.5%	0			
Fataki	45	24	53.3%	5	3	1	4
Gethy	2	1	50.0%	1	1		1
Kambala	2	0	0.0%	0			
Kilo	29	11	37.9%	0			
Komanda	39	29	74.4%	1		2	2
Lita	152	89	58.6%	1			
Logo	10	5	50.0%	0			
Lolwa	7	2	28.6%	0			
Mahagi	4	1	25.0%	0			
Mambasa	10	6	60.0%	0			
Mandima	16	12	75.0%	0			

Mangala	108	67	62.0%	0			
Mongbwalu	561	272	48.5%	3	2	2	4
Nia-Nia	114	67	58.8%	2	1		1
Nizi	437	216	49.4%	6	4	1	5
Nyankunde	115	35	30.4%	0			
Rimba	9	4	44.4%	0			
Rwampara	708	291	41.1%	16	9		9
Tchomia	45	24	53.3%	0			
North Kivu	447	302	67.6%	11	5	5	10
Blessed	65	49	75.4%	1			
Butembo	92	71	77.2%	0		2	2
Goma	1	0	0.0%	0			
Kalunguta	8	4	50.0%	0			
Katwa	208	135	64.9%	8	4	2	6
Kyondo	13	10	76.9%	0			
Lubero	1	1	100.0%	0		1	1
Mabalako	1	0	0.0%	0			
Masereka	5	1	20.0%	0			
Musienene	47	26	55.3%	2	1		1
Oicha	4	3	75.0%	0			
Vuhovi	2	1	50.0%	0			
Haut-Uélé	74	41	55.4%	7	1	1	2
Boma Mangbetu	15	5	33.3%	2			
Gombari	1	1	100.0%	1	1		1
Isiro	26	15	57.7%	1			
Pawa	15	11	73.3%	3			
Rungu	1	0	0.0%	0			
Wamba	16	9	56.2%	0			
Tshopo	8	5	62.5%	1			
Bafwasende	1	0	0.0%	1			
Kabondo	1	1	100.0%	0			
Lubunga	1	0	0.0%	0			
Makiso-Kisangani	3	3	100.0%	0			
Mangobo	1	1	100.0%	0			
Wanie-Rukula	1	0	0.0%	0			
South Kivu	3	1	33.3%	0			
Miti-Murhesa	3	1	33.3%	0			
Total	4053	1,850	45.7%	80	33	16	49

2.3 Mapping of confirmed cases

The spatial distribution confirms that transmission is mainly concentrated in Ituri, in areas already affected, with no new geographical expansion in the last 24 hours. Eleven health zones in Ituri reported confirmed cases on August 5, compared to seven the previous day, but the daily figure is down, from 85 to 80 cases.

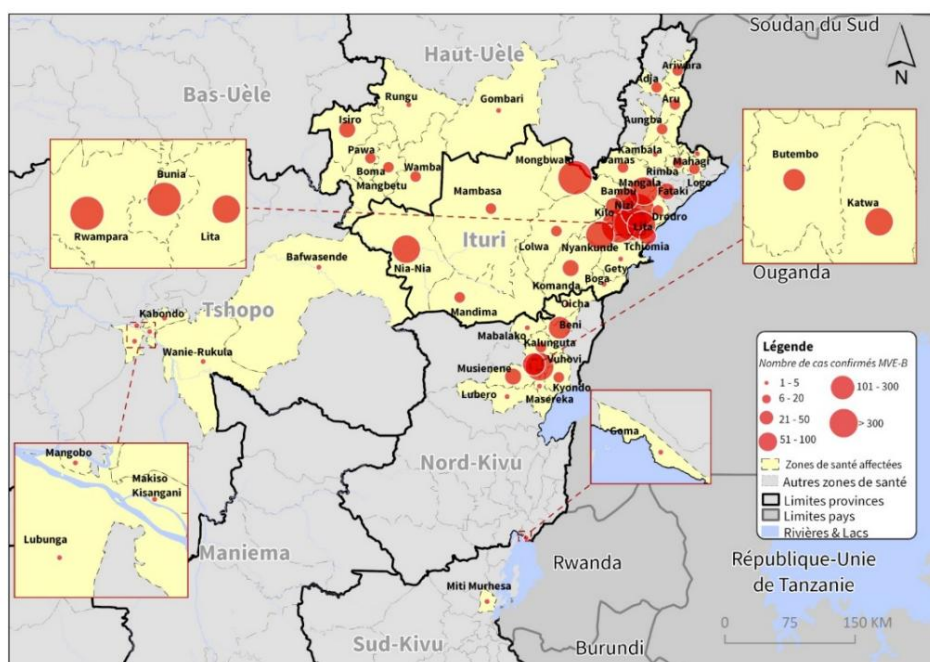


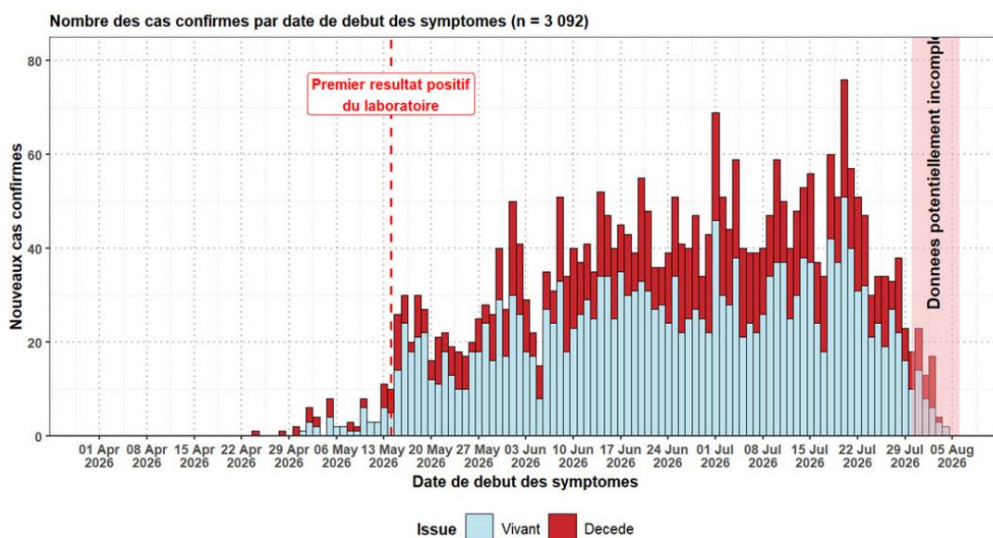
FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission continues in already affected health zones, necessitating simultaneous intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de début de symptômes renseignés, n = 3 092.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of working-age adults (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

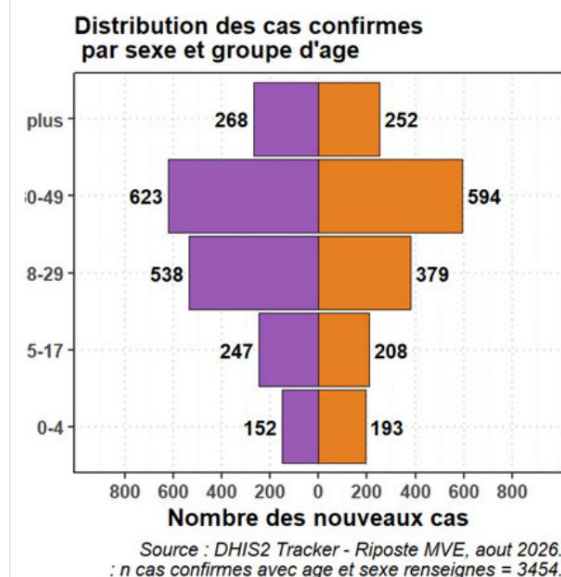
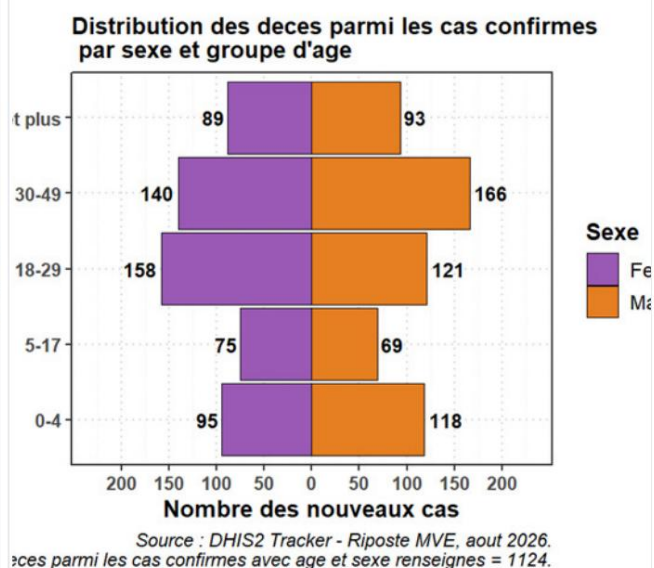


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. In North Kivu, 229 confirmed cases are male and 207 are female, with the 18–29 and 30–49 age groups accounting for 232 of the 436 cases (53.2%). The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases indicates the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu Haut-Uélé	Tshopo	South Kivu	Total
Report (Day -1)	ND	7	ND	0	7
New alerts received	458	772	ND	49	1,281
Total alerts for the day	458	774	ND	49	1,288
Alerts investigated	352	763	ND	41	1,158
Investigation rate (24h)	76.9% 98.6%	ND	83.7% 100.0%		90.3%
Alerts validated—alive	147	53	ND	2	203
Alerts validated—deceased (comment)	46	33	ND	0	79
Totalcassuspectsdujour	193	86	ND	2	282

In total, 1,283 alerts were managed (1,281 of which were new) and 1,158 investigated within 24 hours (90.3%), resulting in 282 confirmed suspected cases (203 living and 79 community deaths). North Kivu accounts for the majority of these with 774 alerts, of which 763 were investigated (98.6%): Beni alone accounts for 245 of these. Ituri managed 458 alerts and investigated 352 within 24 hours (76.9%).

Tshopo reported 49 alerts, 41 of which were investigated (83.7%), while South Kivu reported 2 alerts (100%). Haut-Uélé did not submit an alert table dated August 5th.

3.2 Laboratory: tests and positivity

Ituri : 297 samples received (178 samples from suspected cases, 73 swabs and 46 re-samples), of which 70 were positive — 34 living, 27 deceased and 9 re-samples.

North Kivu: 134 samples received and tested (43 in Beni and 91 in Butembo), of which 11 were positive, including 8 in Katwa, 2 in Musienene and 1 in Beni.

Haut-Uélé: 7 samples tested positive: Pawa (3), Boma Mangbetu (2), Isiro (1) and Gombari (1), including 6 living and 1 death. A confirmed case from the university clinics of Uélé came back negative on the first re-sampling.

Tshopo: 1 sample tested positive for the day (Bafwasende Health Zone); receipt and unpacking of inputs and consumables; briefing of biologists on Ebola virus disease diagnosis and of Red Cross members on biosecurity, swab sampling, and sample transport. The provincial total stands at 8 positive cases.

South Kivu: 5 samples received (Katana 2, Kadutu 1, Ibanda 1 and Bagira 1), all negative. The provincial total stands at 3 positive results out of 300 samples analyzed (1.0%); 94% of samples are analyzed within one day of collection and 5 tests remain pending.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	11763	9324	79.3%
North Kivu*	6352	4882	76.9%
Haut-Uélé**	601	320	53.2%
South Kivu	55	55	100.0%
Tshopo***	115	92	80.0%
Total	18886	14673	77.7%

Contact tracing has risen to 77.7% nationally, compared to 75.3% the previous day, and remains well below the operational threshold of 95%. Ituri is monitoring 9,324 contacts out of 11,763 (79.3%) and has identified 358 new contacts, but two areas account for 66.0% of the 2,439 untraceable contacts: Bunia (818 untraceable, 44.8% follow-up) and Mangala (793 untraceable, 51.2%), followed by Nizi (178) and Fataki (177, no contacts seen). North Kivu has 4,882 contacts seen out of 6,352 (76.9%), with Beni and Katwa accounting for 1,060 of the 1,469 untraceable contacts. Haut-Uélé follows up with 320 of its 601 known contacts (53.2%), Tshopo with 92 out of 115 (80.0%) and Sud-Kivu with all 55 of its contacts (100%).

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 2 alerts notified

Two live alerts were notified at the province's crossing points: detected at the PoC of Mudzibala (Bunia) and Bey (Komanda) were isolated, validated and referred to the dedicated ESS.

North Kivu — 0 alerts notified

Tshopo — not reported

Haut-Uélé — 0 alerts notified

No alerts were notified to the province's activated PoE/PoC devices.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	South Kivu	Tshopo Haut-Uélé	Overall	
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (18/18)	100% (9/9) 21.1% (4/19)	23.7% (9/38)	69.4% (100/144)	
Number of people who have switched to PoE/PoC	106591	ND	0	2278	ND	108869
% of travelers screened	97.2%	97.9%	0	98.6%	ND	97.2%
% of travelers who washed their hands	97.0%	ND	ND	98.2%	ND	97.0%
% of travelers made aware	99.2%	ND	ND	100.0%	ND	99.2%
Number of alerts notified	2	0	0	0	0	2
Number of validated live alerts	2	0	0	0	0	2
Lifeless bodies intercepted today	0	0	0	0	0	0
Problem contacts intercepted today	0	0	0	0	0	0
% of validated live alerts referred to CT/CTE	50.0% (1/2)	0	0	0	0	50.0% (1/2)
% of the lifeless bodies swabbed	ND	ND	0	ND	ND	ND
Positive results from referred suspected cases	0	0	0	0	0	0

4 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that have received interventions	19	5
Confirmed cases are being monitored.	142	7
New suspected cases	21	25
Suspected cases are being monitored	92	66
Laboratory results announced (including positive ones)	53 (19)	56 (19)
Children separated — nursery / community	0 / 45	0 / 18
CTE support staff	309	34
Frontline Personnel (PPL)	43	35
Members of households and communities	55	56
EDS supported (SMSPS)	41	8

5 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD IN ITURI

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Nia-Nia	22	5	12	5
Mongbwalu	18	7	2	9
Nyankunde	14	11	1	2
Lita	4	1	2	1
Mangala	4	2	1	1
Nizi	3	0	0	3
Logo	3	0	3	0
Bamboo	2	2	0	0
Kilo	2	1	1	0
Damascus	1	0	0	1
Total — case fatality rate 30.5%	141	71	24	43

6 Pillar-Based Response Activities

COORDINATION

ŷ Ituri: Coordination meeting and daily pillar meetings; data validation between monitoring, laboratory, and planning section. **ŷ North Kivu:** Support for the coordination of the Musienene Health Zone, briefing of private social economy managers and traditional healers in this area.

ŷ Haut-Uélé: The provincial SGI is operational with regular meetings, implementation of the decree establishing the provincial response plan, and monitoring of the roadmap. Six decisions were made: temporary closure of population movements on the Niania–Wamba axis, establishment of a 21-day quarantine center for passengers on this axis, activation of 33 priority PoCs and 5 PoEs, assignment of 36 security agents to CTEs and laboratories, provision of transport and communication resources, and allocation of 249 Starlink kits (DPS 10, BCZ 13, health centers 213 and HGR 13) with a subscription of at least six months.

CONTINUITY OF ESSENTIAL SERVICES (CSSE)

ŷ Ituri: 29 ambulances, 24 of which were available; 51 trips completed, 50 successful, resulting in 59 patient evacuations; four ambulances remained out of service, and one transfer refusal was recorded in Bambu. The areas with the highest demand were Bunia (15 trips), Nizi (7, for 15 evacuations), Rwampara (5), and Mongbwalu (4), in addition to the 10 trips handled by the call center. The out-of-service ambulances were distributed between the Bambu call center, Mongbwalu, and Rwampara. Other health zones did not report their ambulance movements.

LOGISTICS

ŷ Ituri: delivery of IPC/WASH and EDS kits to the provincial SGI, IPC kits (bleach) to the Nizi Health Zone, and IPC and EDS kits to the Nizi CTE; handover of UNICEF-donated motorcycles to community health workers in the Health Zones from Mambasa and Lita; supply of 603 liters of

ŷ Tshopo: organize a trip to Baego from Kisangani or Bafwasende for the listing and follow-up of contacts around the 8th case (72 hours).	fuel (provincial SGI 190 L, national coordination 413 L); 49 vehicles and 10 ambulances are allocated to the pillars and health zones, including 15 to PCI/EDS, 8 to surveillance, 3 to the provincial laboratory and 3 to the Mongbwalu health zone.
Work continues on finishing the infrastructure for the care facility at the Nia-Nia CTE. ŷ North Kivu: receipt of a donation of IPC kits from WHO from Bunia, awaiting unloading; delivery of nutritional inputs (Plumpy'Nut, F75 and F100) to the health zones of Kyondo, Butembo, Katwa, Musienene, Kalunguta, Vuhovi and Masereka, and of 4 mattresses to Kalunguta as compensation for those incinerated during the decontamination of a household; continuation of construction work on the CTs in Musienene (MSF France) and Beni (MSF Holland); support for the Vuhovi health zone in defining the triage to isolation circuit, its makeshift CTE requiring rehabilitation support. ŷ Tshopo: daily monitoring of stocks and water supply at PoC PK 23; delivery of the first 3 motorcycles by the supplier (17 more expected) ; maintenance of 2 DPS vehicles; prospecting at CTMI and in the field of COUSP with IMC and IMA for the organization of the CTE circuit and start of work on the large capacity site with MSF. ŷ Haut-Uélé: Acquisition of three ambulances as well as equipment and supplies for IPC, patient care, and EDS; provision of fuel for vehicles, motorcycles, and a generator. Rehabilitation work on the road leading to the Isiro CTE has resumed.	PCI / WASH ŷ Ituri: 18 rings opened around cases and 62 sites identified, of which 59 were decontaminated (95%); 35 ESS monitored and supported out of 33 planned (Bunia 21, Mongbwalu 4, Mambasa 3, Nizi 2, Rwampara 2, Mandima 2 and Lita 1), 6 ESS decontaminated and 25 means of transport decontaminated (Mongbwalu 14, Nizi 8, Lita 2 and Bunia 1); 54 households of cases decontaminated within 48 hours out of 95 expected (41% due to insufficient decontamination teams and PPE as well as the lack of replacement household kits); 133 PPLs trained (Nia-Nia 100, Mambasa 21 and Lita 12). ŷ North Kivu: 4 ESSs (healthcare facilities) for patients staying in decontaminated (3 in Katwa and 1 in Musienene); provision of incomplete PCI kits in 3 ESSs (Katwa 2, Musienene 1); 43 ESSs monitored and supported (Oicha 28, Katwa 7, Kyondo 4, Musienene 3 and Vuhovi 1) and 202 providers briefed during formative supervisions (Oicha 134, Katwa 42, Kyondo 19 and Musienene 7); scorecard evaluation of 2 ESSs (Beni 25.8%, Butembo 32.3%) and 3 staff exposure risk assessments, all classified as high risk. ŷ Tshopo: IPC assessment of 2 health facilities (Maman Régine dispensary, SNCC health center, and Anoalilé health center) and follow-up support for these facilities; briefing of 40 ambulance drivers and decontamination workers from urban health zones with support from Momentum; water supply for the Point of Control (POC) at PK 23 on National Road 4. Out of 81 health facilities, only one exceeded 80% on the IPC score (the CTE, 82%), 3 were between 50% and 79%, and 31 were below 50%; 23% (19/81) received IPC kits and 6% (5/81) have a one-way triage system. All health facilities that received a confirmed case were decontaminated within 48 hours (4/4), compared to 50% of households (2/4), and 89% of community deaths (8/9) benefited from a COVID-19 s ŷ Haut-Uélé: training of service providers in Isiro; Decontamination of isolation sites, households, and social and solidarity economy (SSE) facilities; distribution of infection prevention and control (IPC) kits in health zones; and support for zonal teams. The availability of chlorine, other IPC supplies, and water, as well as standardized waste management, remain the main areas for improvement.

COMMUNICATION (RCCE / CREC)

ÿ **Ituri:** 1,959 people reached at home by RECO and 263 during educational talks in the Komanda and Gety health zones, 796 in mass awareness campaigns (Komanda, Rwampara, Gety) and 435 during activities around confirmed cases; 9 people engaged during

2 advocacy meetings. 16 community alerts reported (6 living, 10 dead) in Komanda, Rwampara and Gety; out of 37 feedbacks, 29 were captured and 8 clarified;

Capacity building for 15 community health workers, 8 young influencers and 7 religious leaders in Komanda (MSF, Save the Children), 17 women leaders and 3 local chiefs in Mongbwalu (WHO) and 19 community relays in Lita (Africa CDC); Interactive program produced on RTNC and relayed by 17 partner channels (EPIC/FHI 360); 2 programs and 6 radio spots produced, 36 community radio stations engaged and 15 IEC materials distributed;

Fifteen community radio directors in Bunia have pledged to produce programs in the local language; 5 megaphones and vests were provided to influencers in Rwampara (FHI 360).

ÿ **North Kivu:** motorized caravan in Beni with 100 motorcycle taxi associations; community dialogue chaired by the administrator of the Lubero territory in the Musienene health zone on disinformation, rumor management and the infodemic; briefing of 20 young people from LUCHA concluded with a guided tour of the CTE at the Beni General Referral Hospital; Actions around 21 confirmed cases affecting 95 heads of households (Katwa 9 cases/43 households, Beni 2/14, Butembo 2/10, Kyondo 1/8, Masereka 1/7, Vuhovi 1/7 and Kalunguta 1/6); in Beni, 24 high-risk contacts were listed with the agreement of the families for decontamination; 17 rumors clarified, but none of the 6 expected community incidents in Musienene were resolved.

ÿ **Tshopo:** Deployment of support teams for health zones (13 participants); awareness-raising among 413 households around the Ebola Treatment Center (ETC) under construction and among 546 people during home visits in the Zinia, hospital, and Uniskis blocks (Makiso-Kisangani); support for the training of 60 ambulance drivers for 5 health zones and 60 community stakeholders; advocacy with 8 influencers for the continuation of public health actions related to the case of the motorcyclist from Kabondo and awareness-raising among 30 youth association leaders; continued dissemination of public service announcements. Feedback gathered: "The disease doesn't exist" and "You're fabricating cases to make money."

HOLISTIC CARE (HCC)

ÿ **Ituri:** 15 new recoveries; 468 patients in isolation out of 833 beds (56%), including 271 confirmed and 197 suspected cases, with 55 severe or critical cases; Nutritional support: 1,045 hot meals served, including 760 to contacts, 121 to confirmed cases, 111 to suspected cases, and 53 to people with disabilities, as well as 7 children under 6 months under the LNPE (Law on Nutritional Protection) and 11 children aged 6 to 23 months under SLM.

ÿ **North Kivu:** 8 confirmed cases and 48 suspected cases admitted, no recoveries, 5 deaths and 34 non-cases; 205 patients hospitalized (173 suspected and 32 confirmed). The cumulative total reaches 186 cases admitted to Ebola Treatment Centers (ETCs), 67 deaths and 71 recoveries, for an ETC mortality rate of 36%. Death of the confirmed case in Lubero; Evaluation of the treatment pathway at the Musienene ETC rehabilitated by MSF and briefing of private social economy providers in this area; training of 19 providers from Beni and Oicha in food and nutrition support

with WFP support. On the psychosocial front, 71 people (43 women, 28 men) received psychoeducational sessions in Oicha, Butembo, Kyondo, and Musienene, and 9 separated and orphaned children received support; a strike by service providers of the PEC pillar

psychosocial is announced in Beni.

ÿ **Tshopo:** Two new admissions: the confirmed case from the Baego health center (Bafwasende health zone) transferred to the Kisangani Hospital Center during a joint operation, and a suspected case transferred from Rekapi. The provincial total stands at 6 hospitalized confirmed cases, including 2 deaths, 1 recovery, and 2 currently receiving treatment, one requiring transfer due to comorbidities. Experimental treatments continue for 2 high-risk contacts from Madula (Wanie-Rukula), one of whom was discharged after a single dose, and for 6 staff members at Kisangani Hospital (Day 6). Four patients are occupying the 20 available beds (20%), and one patient has escaped.

ÿ **Haut-Uélé:** training of service providers and medical care of patients in isolation sites; resumption of road rehabilitation work leading to the Isiro CTE.

RESEARCH, INNOVATION & VACCINATION

ÿ **Ituri:** five protocols approved by the ethics committee (100%), 3 out of 5 studies completed (60%), and 4 out of 22 recommendations implemented (22.7%). Ongoing clinical trials: PARTNERS with 60 participants (CTE CME 40, CTE ELIKYA 20) and EBO-PEP with 41 participants (CTE CME 33, CTE ELIKYA 8). Meeting with Africa CDC for planning SBC research pathways, distribution of study recommendations by pillar

(Kabondo), where psychosis is reported among young people close to the 6th case. ÿ Haut-Uélé: training of service providers and community leaders in the 5 affected health zones; holding press briefings; raising awareness through the media and churches; community preparation for the acceptance of EDS and support for other pillars.	rapid qualitative assessment and continued drafting of protocols on laboratory result turnaround times and on the actors involved in the decision-making processes; TDRs developed for the two announced studies.
Dignified and secure burials (DSM) ÿ Ituri: 69 death alerts, 60 death reports (DREs) completed, including 10 in emergency medical centers (EMCs) and 53 bodies identified (49 in the community and 4 in emergency medical services), for a completion rate of 79%; 5 failures and 11 bodies deferred to the following day. Nizi (12 DREs), Mungbwalu (7), Bunia (7), and Rwampara (7) accounted for the majority of the activity, while Nyankunde and Tchomia did not complete any of the reported DREs. The cumulative total has reached 2,914 DREs for 3,583 alerts since May 20. The CREC pillar facilitated the acceptance of 7 DREs in Mangala and Rwampara, as well as in Komanda and Nia-Nia. ÿ North Kivu: 33 death alerts, 33 bodies swabbed and secured, and 30 blood tests performed, with 2 failures due to rushed burials and 11 bodies swabbed pending due to 14 postponements. The alerts mainly originated from Beni (11, or 33.3%), Katwa (10, or 30.3%), and Butembo (6, or 18.1%). The cumulative total stands at 269 community deaths that benefited from a blood test and 440 post-mortem samples.	ÿ Haut-Uélé: 3 community deaths recorded and EDS carried out for the confirmed case of Gombari, imported from PK-51 Niania; Training of service providers and supervision of activities in 4 of the 5 affected health zones, rental of two vehicles in Isiro. ÿ South Kivu: no community deaths or post-mortem swabs were reported for the day; 17 suspects are being treated at CT (Katana 8, Kadutu 5 and Miti-Murhesa 4) and 44 contacts remain in cantonment.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ continuation of community awareness activities on PSEAH and GBV in the affected health zones of Ituri, and continuation of briefing of response actors.	

7 Key challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Community resistance to blood donation and post-mortem sampling, and insecurity targeting teams: attacks on teams in burial areas of North Kivu and inaccessibility of the Bundji cemetery Delayed confirmation, underestimation of cases, and continued chains of transmission around deaths	Intensify CREC, involve religious and community leaders, secure EDS teams, expand ECUMER team coverage and provide adult body bags and PPE; make the Bundji cemetery operational
2	Non-payment and demotivation of striking providers; : service with demonstration by hygiene workers at the CTE Interruption or slowdown of admissions to the CTE and response activities CME of Bunia (tentative resumption), non-payment of some service providers in North Kivu, including police officers assigned to guard the ESS,	Regularize payroll statements (10 validated payroll zones for July in North Kivu), harmonize the benefit scale, and secure dedicated funding.

Challenge No.	Impact	Actions required
	Strike announced by the psychosocial support pillar in Beni, demotivation reported in Tshopo and South Kivu	
3	Insufficient contact tracing performance	Uninterrupted transmission chains
		Train, deploy and motivate community relays (RECO); strengthened supervision; urgent visit to Fataki, Bunia and Mangala; consolidate the observed national recovery (77.7% compared to 75.3% the previous day)
4	Inadequate and immobilized evacuation resources: 4 of the 29 ambulances in Ituri are out of service and 1 transfer was refused in Bambu; there are no ambulances in 7 health zones of North Kivu. and hearses throughout the ZS, lack of means of transport for the supervision of the PoCs of the Tshopo	Delays in transfer and isolation, refusal of transfer
		Repair and pre-position ambulances (including at PoE/PoC), equip the ZS with hearses, deploy CTs by ZS
5	Insufficient IPC supplies (PPE, chlorine) and limited triage coverage: 54 out of 95 households of cases decontaminated in Ituri (41%) and 1 out of 22 in Katwa; 6% of the SSE of the Tshopo (5/81) and 38.1% of those of the North Kivu (109/286) have a unidirectional sorting circuit	pupil infections Nosocomial risks (141 cases of PPL including 43 deaths in Ituri; 14 PPL including 3 deaths at North Kivu; 4 health workers among those confirmed in Haut-Uélé)
		Urgent supply of PPE and chlorine, establishment and training of decontamination teams for health hotspot areas, provision of triage and isolation kits and replacement household kits
6	Geographic expansion: two new health zones reached in 24 hours — Gombari (Haut-Uélé) and Bafwasende (Tshopo) — both linked to the Niania axis; only 9 of the 38 Priority PoE/PoC connections in Haut-Uélé (23.7%) and 4 of the 19 in Tshopo (21.1%) are activated.	Risk of urban, riverine and cross-border spread
		Activate the remaining 29 PoE/PoC points in Haut-Uélé and the 15 in Tshopo; implement the temporary closure of the Niania–Wamba axis and the 21-day quarantine center; strengthen cross-border coordination
7	Insufficient decentralized diagnostic capacities; local laboratories are being installed (Rwampara, Fataki, Bambu-mine, Boga and Mambasa); At Haut-Uélé, machine installed in Isiro and 7 Radi One acquired but analyses not decentralized at the zonal level	Long confirmation delays, transmission not detected
		Accelerate the deployment of decentralized capacities, finance the transport of samples, and equip sites with inverters and sampling vehicles.
8	Inadequacy of the resources financial and non-compliance with commitments by certain partners in the health zones of North Kivu	Limitation of the implementation of the pillars
		Urgent mobilization of resources, advocacy with partners and sharing of financial information from development partners
9	Insecurity and limited access (CODECO, ADF); recurring attacks on response teams in certain health areas of North Kivu and refusal of security personnel to intervene at sites managed by MSF	Restriction of operations
		Strengthen security coordination and humanitarian access; map safe and at-risk health areas; hold a working session with MSF on site security
10	Continuity of essential services (CSSE) compromised	Increase in indirect (non-Ebola) mortality and loss of visibility on healthcare provision
		Effective implementation of free healthcare, strengthening of the CSSE and reintegration

Challenge No.	Impact	Actions required
11 Insufficient reception capacity of CTE/CT	Congestion of facilities, isolation delays and late data reporting	systematic approach to the section in provincial presentations Accelerate the expansion work on the Nizi CTEs, Lita, Fataki, Nia-Nia and Logo, build the missing CTs (including a 10-bed CTE in Kisangani) and equip the CTEs/CTs with Starlink kits

8 Photos of field activities

End of the capacity-building workshop for 32 members of the SMSPS Working Group (15 women) on the implementation of the minimum set of SMSPS services in the Ebola response and the completion of 4 W



FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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